



CI Institute of
Nursing

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BRN Case Management CE Package - Draft / Pending BRN CEP Approval

BRN Care Manager / RN Case Management CEP | Draft / Pending BRN CEP Approval

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Provider (from master packet): CI Institute of Nursing, Inc. - 419 E. Hamilton Ave., Campbell, CA 95008 - 650-449-6706 - humanresources@ci-ion.com - <https://www.ci-ion.com> **Working course title:** RN Case Management in Home Health: Care Coordination, Documentation, Patient Advocacy, and Safe Transitions of Care **Lane scope:** California BRN Continuing Education Provider (CEP) lane only - RN Case Management / RN Care Manager CE. **Approval status:** Draft / Pending BRN CEP Approval. No CEP number has been issued. **Document type:** LMS course package + application-support draft. Not an advertisement, not a certificate, not an approval claim.

Required placeholder statement (applies to every artifact in this package): "Pending California Board of Registered Nursing Continuing Education Provider approval. Do not advertise, issue certificates, or represent this course as approved until a BRN CEP number has been issued."

Program-structure note - read first. The BRN / CEP lane is a **30-contact-hour, 10-module RN case management continuing education program** (10 modules x 3 contact hours each; 50 instructional minutes = 1 contact hour; minimum 1,500 instructional minutes, 1,680 design minutes). The official BRN contact-hour map (02_BRN_CEP_COURSE_AND_CONTACT_HOUR_MAP.md) and BRN_TRANSFER_WORKSHEET.md are aligned to this 30-contact-hour structure. Any earlier 4-contact-hour representative course is **superseded** and is **not part of the current BRN CEP signer-review package**. Approval posture: pending BRN CEP approval (provider approval, not course-by-course approval). **[[SME REVIEW REQUIRED BEFORE SUBMISSION]]**

A. Executive Summary

This package is a build-ready blueprint for one California-BRN-renewal-compatible continuing education course for registered nurses working in care/case management. It is sized at **30 contact hours**, organized into **10 modules of 3 contact hours each**, and engineered for delivery in **Moodle** with auditable completion evidence.

- **Contact-hour math (draft):** 30 contact hours x 50 instructional minutes = **1,500 instructional minutes minimum**. Each module = 3 contact hours = **≥150 instructional minutes**. No module is under 60 minutes. This is design math only, **not** an approval claim.
- **BRN fit posture:** Every module is mapped to a *direct or indirect patient/client-care* rationale grounded in the practice of registered nursing (care coordination, assessment, documentation, advocacy, transitions, utilization review, quality/risk, chronic/geriatric management). See Section C.
- **Primary deliverable:** this markdown course package. The Excel workbook (BRN_CM_CE_LMS_BUILD_WORKBOOK.xlsx) is a **support/defensibility artifact only** (Section Q).

- **What this course is NOT:** It does **not** grant a case management certification, CCM credential, social-work license credit, or any scope expansion. It is RN continuing education only.
- **Source of truth:** Provider identity, instructor of record, policies, certificate elements, and fee are taken from the master packet. Course-design content (modules, lessons, assessments) is **new draft instructional design** and is marked for SME/compliance review throughout.
- **Readiness:** Instructional design is implementation-ready; **regulatory promotion is gated** on SME approval of the 30-hour scope, BRN CEP number issuance, and the verification items in 06_MISSING_ITEMS.md / 08_DO_NOT_SUBMIT_UNTIL.md.

B. Compliance Guardrails

These guardrails are binding on every artifact, screen, certificate, and advertisement produced from this package.

#	Guardrail	Why	Enforced where
B1	No approval language until a BRN CEP number is issued. Use the placeholder statement.	BRN approves providers, not pre-approved claims.	Flyer, certificate, course landing page, evaluation
B2	Use "contact hours," never "CEUs," never "approved for X hours" (except as placeholder).	Matches packet flyer control and BRN terminology.	All learner-facing copy
B3	Each contact hour = ≥50 minutes of organized instruction; no module < 60 minutes.	BRN contact-hour rule.	Course map, module minutes, Moodle time evidence
B4	Certificate must carry a 4-year retention statement and no blank certificates.	Packet policy + BRN recordkeeping.	Certificate (Section N), certificate gate (Section J)
B5	Provider records retained ≥4 years (institutional posture: 5 years).	Packet POLICIES_PACKET.md.	Recordkeeping plan (Section P)
B6	No PHI / no real patient, resident, family, employer, or facility data. All cases fictional and de-identified.	Privacy + packet directive.	Every scenario, every exercise, Moodle guardrails
B7	No certification / scope-expansion claims (no CCM, no LCSW credit, no licensure).	Avoid misrepresentation.	Flyer, landing page, objectives, certificate
B8	80% minimum pass threshold; no learner-visible answer key; separate internal key.	Assessment integrity.	Quiz settings, gradebook, admin key
B9	Implicit-bias reflection is woven into communication/documentation/SDOH content.	Mirrors packet course-information field.	Modules 2, 5, 9; capstone
B10	Anything uncertain is marked "Needs confirmation"; nothing about the packet is invented.	Source-of-truth discipline.	Sections K, L, T
B11	Fictional clinical content is educational, not medical advice or a standing order.	Risk control.	Module intros, scenario headers

C. BRN Fit Rationale

Standard applied: BRN CE content must be relevant to the practice of registered nursing and related to scientific knowledge, technical skills, or direct/indirect patient/client care. Nursing administration/management, quality assurance, legal aspects of nursing, current healthcare trends, care coordination, patient advocacy, documentation, discharge planning, and utilization/risk management qualify when **clearly tied to nursing practice**. This course ties every module to that standard.

Module	Direct or indirect patient care	One-line nursing-practice anchor
1. Foundations & Scope	Indirect	Defines RN scope/role boundaries that govern safe nurse-led case management.
2. Assessment, Intake & Care Planning	Direct	Nursing assessment and patient-centered care planning are core RN practice.
3. Care Coordination Across Disciplines/Settings	Direct + Indirect	Nurse-led interdisciplinary coordination protects continuity of patient care.
4. Documentation & Legal Charting	Indirect	Accurate nursing documentation is a legal/clinical patient-safety function.
5. Patient Rights, Consent, Advocacy, Ethics	Direct	Advocacy, informed consent, ethical boundaries are direct RN duties.
6. Transitions & Discharge Planning	Direct	Discharge planning prevents readmission harm - direct patient care.
7. Utilization Review & Medical Necessity	Indirect	UR tied to nursing judgment protects appropriate, safe patient care.
8. Quality, Risk, Incident Reporting, Escalation	Indirect	QA/risk/escalation are nursing patient-safety and legal-aspects content.
9. Chronic, Geriatric & High-Risk Management	Direct	Applies scientific knowledge to high-risk patient populations.
10. Integrated Capstone & Final Assessment	Direct + Indirect	Synthesizes nurse-led care management across the patient journey.

Defensibility note: Modules 7 and 8 (UR / QA-risk) are the highest-scrutiny items because they border administrative content. Each is written to keep the **nursing-judgment and patient-safety thread** explicit so it reads as indirect patient care, not pure business administration. [[SME REVIEW REQUIRED BEFORE SUBMISSION]]

D. 30-Contact-Hour Course Map

Contact-hour rule: 50 instructional minutes = 1 contact hour. Estimated learner minutes include reading, scenario work, knowledge checks, reflection, documentation practice, and quiz time, and meet or exceed the 150-minute floor per module.

Module ID	Title	Contact hrs	Est. learner min	Direct/Indirect	Completion requirement	Evidence artifact
CM-M01	Foundations of RN Case Management and Nursing Scope	3	165	Indirect	All activities complete + KC ≥80%	Activity-completion log, KC score
CM-M02	Assessment, Intake, and Patient-Centered Care Planning	3	170	Direct	All activities + intake exercise submitted + KC ≥80%	Care-plan worksheet, KC score
CM-M03	Care Coordination Across Disciplines and Settings	3	165	Direct/Indirect	All activities + coordination map + KC ≥80%	Interdisciplinary map artifact
CM-M04	Documentation, Legal Charting, and Audit-Ready Records	3	170	Indirect	All activities + charting exercise + KC ≥80%	De-identified chart-note sample
CM-M05	Patient Rights, Consent, Advocacy, and Ethical Boundaries	3	165	Direct	All activities + ethics reflection + KC ≥80%	Reflection submission, KC score
CM-M06	Transitions of Care, Discharge Planning, and Community Resources	3	170	Direct	All activities + discharge plan + KC ≥80%	Discharge-plan artifact

Module ID	Title	Contact hrs	Est. learner min	Direct/Indirect	Completion requirement	Evidence artifact
CM-M07	Utilization Review, Medical Necessity, and Payer-Safe Communication	3	165	Indirect	All activities + necessity rationale + KC ≥80%	Medical-necessity rationale note
CM-M08	Quality Assurance, Risk Management, Incident Reporting, and Escalation	3	165	Indirect	All activities + incident report + KC ≥80%	De-identified incident report
CM-M09	Chronic Disease, Geriatric, and High-Risk Patient Management	3	170	Direct	All activities + risk-stratification exercise + KC ≥80%	Risk-stratification worksheet
CM-M10	Integrated Case Management Capstone and Final Assessment	3	175	Direct/Indirect	Capstone submitted + final exam ≥80%	Capstone package + final-exam record
Total	10 modules	30	1,680	-	All modules complete + final ≥80%	Full course audit export

Estimated total learner time (1,680 min) exceeds the 1,500-minute instructional floor, providing buffer for time-on-task evidence.

E. Detailed Module Blueprints

Each blueprint below includes: Module ID, title, contact hours, estimated minutes, BRN rationale, measurable objectives, lesson breakdown, LMS activity sequence, knowledge checks, case scenario, documentation/decision exercise, completion requirement, evidence artifact, SME/compliance flags, PHI-safety notes, accessibility notes, and suggested Moodle activity types. **All scenarios are fictional and de-identified. No PHI.**

CM-M01 - Foundations of RN Case Management and Nursing Scope

- **Contact hours / est. minutes:** 3 / 165
- **BRN content rationale (indirect patient care):** Establishes the RN scope-of-practice and role boundaries that make nurse-led case management safe and lawful; frames case management as a function within registered-nursing practice rather than a separate license.
- **Learning objectives (measurable):** By the end of the module the learner will be able to:
 - 1 Define RN case management and distinguish it from social work and unlicensed care coordination, using BRN scope language.
 - 2 Identify three RN scope-of-practice boundaries that constrain case-management decisions.
 - 3 Describe how care coordination qualifies as direct/indirect patient care under nursing practice.
 - 4 Recognize when a case-management task must be escalated to a licensed prescriber or RN supervisor.
- **Lesson breakdown:**
 - L1 What RN case management is (and is not) - 35 min
 - L2 Nursing scope of practice and legal boundaries - 40 min
 - L3 Roles on the care team; RN vs. SW vs. unlicensed staff - 35 min
 - L4 The case-management process model (screen → assess → plan → coordinate → monitor → evaluate) - 35 min

- Knowledge check + reflection - 20 min
- **LMS activity sequence:** Page (overview) → Book/SCORM lesson (L1-L4) → Quiz (KC1) → Assignment (reflection) → Activity completion checkpoint.
- **Knowledge checks (low-stakes):** 8-item KC1 (MCQ + true/false), unlimited attempts, ≥80% to complete; immediate per-item feedback, no full key.
- **Case scenario (fictional):** "Nurse R receives a home-health referral for a fictional 72-year-old client recovering from a hip procedure. The referral asks Nurse R to 'adjust the pain medication.'" Learner identifies the scope boundary and the correct escalation path.
- **Documentation/decision exercise:** Learner writes a 5-line scope-and-escalation note documenting why the medication change is out of RN scope and who it was routed to.
- **Completion requirement:** All lessons viewed + KC1 ≥80% + reflection submitted.
- **Evidence artifact:** Activity-completion log, KC1 score, reflection text.
- **SME/compliance flags:** Confirm BRN scope phrasing with RN SME (Vanessa Valerio). [[SME REVIEW REQUIRED]]
- **PHI-safety notes:** Scenario fully fictional; no names, MRNs, dates of service, or facilities.
- **Accessibility notes:** Captioned video, text transcript, semantic headings, no color-only cues, alt text on diagrams.
- **Suggested Moodle activity types:** Page, Book, Quiz, Assignment, Label.

CM-M02 - Assessment, Intake, and Patient-Centered Care Planning

- **Contact hours / est. minutes:** 3 / 170
- **BRN content rationale (direct patient care):** Nursing assessment, problem identification, and patient-centered goal setting are core RN clinical practice and the foundation of safe care plans.
- **Learning objectives:** The learner will be able to:
 - 1 Conduct a structured intake using a standardized nursing assessment framework.
 - 2 Differentiate subjective and objective data and document each appropriately.
 - 3 Write at least two measurable, patient-centered goals from assessment findings.
 - 4 Identify social determinants of health and screen for implicit bias in intake judgments.
- **Lesson breakdown:** L1 Intake workflow & consent-to-assess (35) · L2 Nursing assessment frameworks/SDOH (40) · L3 From data to nursing problems (35) · L4 Writing measurable, patient-centered goals (40) · KC + exercise (20).
- **LMS activity sequence:** Page → Book lesson → Database/Glossary (assessment terms) → Quiz (KC2) → Assignment (care-plan worksheet) → checkpoint.
- **Knowledge checks:** 10-item KC2; scenario-tagged items; ≥80%.
- **Case scenario (fictional):** Fictional new home-health client with diabetes and limited transportation. Learner extracts subjective/objective data and flags an SDOH barrier.
- **Documentation/decision exercise:** Complete a **Care-Plan Worksheet** (template provided): 3 problems, 2 measurable goals, 1 SDOH intervention, plus a one-line bias-check note.
- **Completion requirement:** Lessons + KC2 ≥80% + worksheet submitted.
- **Evidence artifact:** Care-plan worksheet, KC2 score.
- **SME/compliance flags:** Validate care-plan template against current nursing-process standards. [[SME REVIEW REQUIRED]]
- **PHI-safety notes:** Template pre-loaded with fictional data; field-level reminder "Fictional data only - no PHI."

- **Accessibility notes:** Form fields labeled for screen readers; downloadable accessible template (DOCX + tagged PDF).
- **Suggested Moodle activity types:** Page, Book, Glossary, Quiz, Assignment, Database.

CM-M03 - Care Coordination Across Disciplines and Settings

- **Contact hours / est. minutes:** 3 / 165
- **BRN content rationale (direct + indirect):** Nurse-led interdisciplinary coordination protects continuity and prevents gaps that cause patient harm.
- **Learning objectives:** The learner will be able to:
 - 1 Map an interdisciplinary team and each member's contribution to a care plan.
 - 2 Apply structured handoff communication (e.g., SBAR) to a coordination event.
 - 3 Identify two failure points in cross-setting transitions and a nursing mitigation for each.
 - 4 Distinguish information that may be shared under minimum-necessary principles.
- **Lesson breakdown:** L1 The interdisciplinary team & RN's coordinating role (35) · L2 Structured communication/SBAR (40) · L3 Cross-setting coordination: home health, primary care, specialty, payer (35) · L4 Closing the loop & follow-up tracking (35) · KC + map (20).
- **LMS activity sequence:** Page → Lesson (branching scenario) → Wiki/Database (team map) → Quiz (KC3) → Assignment (coordination map) → checkpoint.
- **Knowledge checks:** 8-item KC3 + 1 branching Lesson decision path.
- **Case scenario (fictional):** Fictional client discharged from a hospital to home health with a missing medication reconciliation. Learner sequences the coordination steps.
- **Documentation/decision exercise:** Build an **Interdisciplinary Coordination Map** (who/what/when/loop-closure) and a 3-line SBAR.
- **Completion requirement:** Lessons + KC3 ≥80% + coordination map submitted.
- **Evidence artifact:** Coordination map, SBAR text, KC3 score.
- **SME/compliance flags:** Confirm SBAR/handoff examples match current practice. [[SME REVIEW REQUIRED]]
- **PHI-safety notes:** All team/member names fictional; no real facility names.
- **Accessibility notes:** Branching Lesson keyboard-navigable; map template accessible.
- **Suggested Moodle activity types:** Lesson (branching), Wiki, Database, Quiz, Assignment.

CM-M04 - Documentation, Legal Charting, and Audit-Ready Records

- **Contact hours / est. minutes:** 3 / 170
- **BRN content rationale (indirect patient care; legal aspects of nursing):** Accurate, timely, defensible documentation is a patient-safety and legal function of nursing practice.
- **Learning objectives:** The learner will be able to:
 - 1 Apply documentation standards (accurate, objective, timely, complete, legible/attributable).
 - 2 Correct a non-compliant chart entry using accepted late-entry/error-correction conventions.
 - 3 Identify documentation that would and would not withstand an audit.
 - 4 Explain retention requirements and the four-year certificate/record posture.

- **Lesson breakdown:** L1 Purpose & legal weight of the record (35) · L2 Standards & charting do/don't (40) · L3 Error correction, late entries, addenda (35) · L4 Audit-readiness & retention (35) · KC + exercise (25).
- **LMS activity sequence:** Page → Book → Quiz (KC4) → Assignment (chart-note rewrite) → checkpoint.
- **Knowledge checks:** 10-item KC4 incl. "spot the documentation error" items.
- **Case scenario (fictional):** A fictional vague chart note ("pt seems fine, will follow up") that the learner must rewrite to standard.
- **Documentation/decision exercise:** Rewrite the weak note into a compliant, audit-ready, de-identified entry and add a properly formatted correction/addendum.
- **Completion requirement:** Lessons + KC4 ≥80% + rewritten note submitted.
- **Evidence artifact:** De-identified chart-note sample, KC4 score.
- **SME/compliance flags:** Legal-aspects content reviewed by RN SME + compliance. [[SME REVIEW REQUIRED]] [[LEGAL REVIEW]]
- **PHI-safety notes:** Provided notes contain only fictional placeholders; learners instructed never to paste real records.
- **Accessibility notes:** Before/after examples in text (not images of text); screen-reader friendly.
- **Suggested Moodle activity types:** Page, Book, Quiz, Assignment.

CM-M05 - Patient Rights, Consent, Advocacy, and Ethical Boundaries

- **Contact hours / est. minutes:** 3 / 165
- **BRN content rationale (direct patient care):** Advocacy, informed consent, confidentiality, and ethical boundaries are direct nursing duties protecting patients.
- **Learning objectives:** The learner will be able to:
 - 1 Describe core patient rights relevant to care management.
 - 2 Distinguish informed consent, assent, and refusal and the RN's role in each.
 - 3 Apply an ethical decision-making framework to a boundary conflict.
 - 4 Identify implicit-bias risks in advocacy decisions and a mitigation.
- **Lesson breakdown:** L1 Patient rights & confidentiality (35) · L2 Informed consent/refusal (35) · L3 Advocacy in practice (35) · L4 Ethical boundaries & conflicts of interest (40) · KC + reflection (20).
- **LMS activity sequence:** Page → Book → Choice (ethics poll) → Quiz (KC5) → Assignment (ethics reflection) → checkpoint.
- **Knowledge checks:** 8-item KC5 + 1 ethics scenario item.
- **Case scenario (fictional):** A fictional client refuses a recommended service the family wants; learner navigates advocacy vs. autonomy.
- **Documentation/decision exercise:** Ethics reflection (250-350 words) applying a named framework + documenting the consent/refusal conversation in note form.
- **Completion requirement:** Lessons + KC5 ≥80% + reflection submitted.
- **Evidence artifact:** Reflection submission, KC5 score.
- **SME/compliance flags:** Confirm consent/advocacy content; ensure no scope/legal-advice overreach. [[SME REVIEW REQUIRED]] [[LEGAL REVIEW]]
- **PHI-safety notes:** Fictional family/client; reflection rubric forbids real cases.
- **Accessibility notes:** Choice/poll accessible; reflection accepts assistive-tech input.

- **Suggested Moodle activity types:** Page, Book, Choice, Quiz, Assignment.

CM-M06 - Transitions of Care, Discharge Planning, and Community Resources

- **Contact hours / est. minutes:** 3 / 170
- **BRN content rationale (direct patient care):** Effective discharge planning and warm handoffs reduce avoidable readmissions and patient harm.
- **Learning objectives:** The learner will be able to:
 - 1 Build a discharge/transition plan addressing meds, follow-up, equipment, and caregiver needs.
 - 2 Apply teach-back to confirm patient/caregiver understanding.
 - 3 Match at least three community resources to identified needs.
 - 4 Identify red-flag gaps that should delay or modify a transition.
- **Lesson breakdown:** L1 Transition risk & readmission drivers (35) · L2 Discharge-plan components (40) · L3 Teach-back & caregiver readiness (35) · L4 Community resources & referrals (35) · KC + plan (25).
- **LMS activity sequence:** Page → Book → Database (resource directory, fictional) → Quiz (KC6) → Assignment (discharge plan) → checkpoint.
- **Knowledge checks:** 10-item KC6.
- **Case scenario (fictional):** Fictional client going home with new equipment and a caregiver who works full time. Learner identifies gaps and resources.
- **Documentation/decision exercise:** Complete a **Discharge/Transition Plan** template incl. teach-back confirmation line.
- **Completion requirement:** Lessons + KC6 ≥80% + discharge plan submitted.
- **Evidence artifact:** Discharge-plan artifact, KC6 score.
- **SME/compliance flags:** Validate teach-back & resource categories. [[SME REVIEW REQUIRED]]
- **PHI-safety notes:** Resource directory uses fictional/generic resource types, no real client data.
- **Accessibility notes:** Plan template accessible; resource list in readable table.
- **Suggested Moodle activity types:** Page, Book, Database, Quiz, Assignment.

CM-M07 - Utilization Review, Medical Necessity, and Payer-Safe Communication

- **Contact hours / est. minutes:** 3 / 165
- **BRN content rationale (indirect patient care; current healthcare trends):** Utilization review tied to nursing judgment ensures patients receive appropriate, safe, non-excessive care; payer-safe communication protects the patient and the record.
- **Learning objectives:** The learner will be able to:
 - 1 Define medical necessity and the RN's role in supporting (not determining coverage of) it.
 - 2 Apply criteria-based review to a fictional service request.
 - 3 Write a defensible medical-necessity rationale in nursing terms.
 - 4 Communicate with payers without misrepresentation or scope overreach.
- **Lesson breakdown:** L1 What UR is and the RN's lane (40) · L2 Medical-necessity criteria & evidence (40) · L3 Documentation that supports necessity (35) · L4 Payer-safe, accurate communication (30) · KC + rationale (20).

- **LMS activity sequence:** Page → Book → Quiz (KC7) → Assignment (necessity rationale) → checkpoint.
- **Knowledge checks:** 8-item KC7.
- **Case scenario (fictional):** Fictional request for extended home-health visits; learner reviews against criteria and drafts a rationale.
- **Documentation/decision exercise:** Write a **Medical-Necessity Rationale** note (clinical facts → criteria → conclusion) with a payer-safe communication snippet.
- **Completion requirement:** Lessons + KC7 ≥80% + rationale submitted.
- **Evidence artifact:** Medical-necessity rationale note, KC7 score.
- **SME/compliance flags: High-scrutiny module** - keep nursing-judgment/patient-safety thread explicit so content reads as indirect patient care, not business administration. [[SME REVIEW REQUIRED]] [[COMPLIANCE REVIEW]]
- **PHI-safety notes:** Fictional request data only.
- **Accessibility notes:** Criteria tables screen-reader friendly.
- **Suggested Moodle activity types:** Page, Book, Quiz, Assignment.

CM-M08 - Quality Assurance, Risk Management, Incident Reporting, and Escalation

- **Contact hours / est. minutes:** 3 / 165
- **BRN content rationale (indirect patient care; quality assurance + legal aspects):** QA, risk identification, incident reporting, and escalation are nursing patient-safety functions.
- **Learning objectives:** The learner will be able to:
 - 1 Identify quality indicators relevant to case management.
 - 2 Recognize a reportable event and complete a compliant incident report.
 - 3 Apply an escalation pathway (who, when, how) to a safety concern.
 - 4 Distinguish quality improvement from blame and describe a just-culture response.
- **Lesson breakdown:** L1 QA fundamentals & indicators (35) · L2 Risk identification (35) · L3 Incident reporting standards (40) · L4 Escalation & just culture (35) · KC + report (20).
- **LMS activity sequence:** Page → Book → Quiz (KC8) → Assignment (incident report) → checkpoint.
- **Knowledge checks:** 10-item KC8.
- **Case scenario (fictional):** Fictional near-miss medication discrepancy discovered during a home visit; learner reports and escalates.
- **Documentation/decision exercise:** Complete a de-identified **Incident Report** + escalation note.
- **Completion requirement:** Lessons + KC8 ≥80% + incident report submitted.
- **Evidence artifact:** De-identified incident report, KC8 score.
- **SME/compliance flags: High-scrutiny module** - anchor to patient-safety/nursing practice, not pure administration. [[SME REVIEW REQUIRED]] [[COMPLIANCE REVIEW]]
- **PHI-safety notes:** Incident template uses fictional data; explicit no-PHI banner.
- **Accessibility notes:** Report template accessible; instructions in plain language.
- **Suggested Moodle activity types:** Page, Book, Quiz, Assignment.

CM-M09 - Chronic Disease, Geriatric, and High-Risk Patient Management

- **Contact hours / est. minutes:** 3 / 170

- **BRN content rationale (direct patient care; scientific knowledge):** Applies clinical/scientific knowledge to manage high-risk populations and reduce decompensation.
- **Learning objectives:** The learner will be able to:
 - 1 Apply risk-stratification to identify high-risk patients in a panel.
 - 2 Describe nursing management priorities for common chronic conditions in care management.
 - 3 Identify geriatric-specific risks (falls, polypharmacy, cognition) and nursing mitigations.
 - 4 Build a monitoring plan with escalation triggers.
- **Lesson breakdown:** L1 Risk stratification (40) · L2 Chronic-disease management priorities (40) · L3 Geriatric syndromes & polypharmacy (35) · L4 Monitoring plans & triggers (35) · KC + exercise (20).
- **LMS activity sequence:** Page → Book → Quiz (KC9) → Assignment (risk-stratification worksheet) → checkpoint.
- **Knowledge checks:** 10-item KC9.
- **Case scenario (fictional):** Fictional 80-year-old with multiple chronic conditions and 9 medications; learner stratifies risk and sets monitoring triggers.
- **Documentation/decision exercise:** Complete a **Risk-Stratification + Monitoring Worksheet** with at least three escalation triggers.
- **Completion requirement:** Lessons + KC9 ≥80% + worksheet submitted.
- **Evidence artifact:** Risk-stratification worksheet, KC9 score.
- **SME/compliance flags:** Confirm clinical content currency; mark as education, not medical advice. [[SME REVIEW REQUIRED]]
- **PHI-safety notes:** Fictional clinical profile only.
- **Accessibility notes:** Worksheet and tables accessible; avoid color-only risk tiers (use labels).
- **Suggested Moodle activity types:** Page, Book, Quiz, Assignment.

CM-M10 - Integrated Case Management Capstone and Final Assessment

- **Contact hours / est. minutes:** 3 / 175
- **BRN content rationale (direct + indirect):** Synthesizes nurse-led care management across the full patient journey and verifies competency.
- **Learning objectives:** The learner will be able to:
 - 1 Integrate assessment, coordination, documentation, advocacy, transitions, and QA into one case.
 - 2 Produce an audit-ready, de-identified case-management documentation package.
 - 3 Demonstrate ≥80% mastery on a cumulative final assessment.
 - 4 Reflect on implicit-bias and ethical considerations in the integrated case.
- **Lesson breakdown:** L1 Capstone briefing & rubric walkthrough (30) · L2 Building the integrated case package (60) · L3 Self-review against rubric (25) · Final exam (60).
- **LMS activity sequence:** Page (briefing) → Assignment (capstone package, rubric-graded) → Quiz (final exam, gated to ≥80%) → Certificate gate.
- **Knowledge checks:** Capstone rubric self-check; final exam is the cumulative assessment (Section H).
- **Case scenario (fictional):** A fictional end-to-end home-health case from intake to safe transition; learner produces the full documentation package.

- **Documentation/decision exercise:** Capstone package (Section I): care plan + coordination map + discharge plan + necessity rationale + incident/escalation note + bias/ethics reflection.
- **Completion requirement:** Capstone submitted + meets rubric minimum + final exam $\geq 80\%$.
- **Evidence artifact:** Capstone package + final-exam attempt record.
- **SME/compliance flags:** Capstone rubric and final exam blueprint require SME sign-off. [[SME REVIEW REQUIRED]] [[COMPLIANCE REVIEW]]
- **PHI-safety notes:** Capstone instructions forbid real cases; automated reminder + manual review for PHI.
- **Accessibility notes:** Extended-time accommodation supported; rubric provided in accessible format.
- **Suggested Moodle activity types:** Assignment (with rubric), Quiz, Page, Certificate (gated).

F. LMS / Moodle Build Plan

Course shell

- **Format:** Topics format, one section per module (10 sections) + a Section 0 "Start Here" and a final "Certificate & Records" section.
- **Course full name:** RN Case Management in Home Health: Care Coordination, Documentation, Patient Advocacy, and Safe Transitions of Care - *Draft / Pending BRN CEP Approval*.
- **Course summary:** Carries the placeholder approval statement and the "no certification / no scope expansion" notice.

Sections / topics layout

- Section 0 - Start Here: orientation, no-PHI policy, accessibility statement, how completion works, academic-integrity notice.
- Sections 1-10 - Modules CM-M01...CM-M10 (each: lessons, KC quiz, assignment, completion checkpoint).
- Final Section - Final exam recap, evaluation form, certificate (gated), records notice.

Completion tracking configuration

- Enable course **Completion tracking**; set activity completion per item.
- Lessons/Books: "require view." Quizzes: "require passing grade ($\geq 80\%$).\" Assignments: "require submission" (+ "require grade" where faculty-scored).
- Course completion = all 10 module checkpoints + final exam passed + evaluation submitted.

Restrict access rules (sequential gating)

- Each module section restricted by completion of the prior module (Restrict access $\rightarrow$ activity completion).
- Final exam restricted until Modules 1-9 complete and capstone submitted.
- Certificate restricted until course completion criteria met (Section J).

Quiz settings

- KCs: unlimited attempts, highest grade, immediate feedback, **no review of correct answers after close**, question + answer shuffling, $\geq 80\%$ to complete.
- Final exam: limited attempts (recommend 3), question bank random draw, shuffle, **no review showing the full answer key**, time limit on, $\geq 80\%$ to pass.

Attempt settings

- KC remediation: after a failed attempt, show per-item feedback + a "review this lesson" link, then allow retry.

- Final exam: after each failed attempt, route to a remediation page (Section G) before next attempt; lock after max attempts → manual faculty review.

Passing threshold

- 80% on all graded knowledge checks and the final exam; capstone must meet rubric minimum.

Gradebook structure (categories)

- Knowledge Checks (10 items, low weight), Applied Exercises/Assignments (10 items), Capstone (rubric), Final Exam (gate). Course total configured so certificate logic reads "complete + passed."

Certificate gate conditions - see Section J.

Activity completion conditions - per-item as above; every module ends in a visible completion checkpoint (Label/Page) so learners see progress.

Time / participation evidence strategy

- Use Moodle logs + activity completion timestamps + quiz attempt durations as time-on-task evidence.
- Minimum dwell where supported (Lesson/SCORM timing); discourage instant click-through.
- Export completion report per learner as the participation record (Section P).

Learner progress dashboard recommendations

- Enable the Completion progress / progress-bar block + the Course completion status block on the course page; learner-facing "what's left" view.

Mobile-first design notes

- Single-column responsive layout; tap targets ≥44px; avoid wide tables in lessons; chunk lessons into short pages; ensure templates downloadable and editable on mobile; test in Moodle mobile app.

Accessibility requirements (WCAG 2.1 AA target)

- Captions + transcripts for all media; semantic headings; alt text; sufficient contrast; keyboard navigation; no color-only meaning; accessible quiz question formats; accessible downloadable templates (tagged PDF + DOCX); extended-time accommodation supported.

No-PHI guardrails

- Site/course banner: "Educational content only. Do not enter real patient/resident data (no PHI)."
- Assignment intros + submission reminders repeat the no-PHI rule; faculty PHI spot-check before grading; capstone manual PHI review.

Audit evidence export plan - see Section P.

G. Assessment Plan

Layer	Instrument	Purpose	Threshold	Learner sees	Admin/faculty sees
1	Module knowledge checks (KC1-KC9 + capstone self-check)	Low-stakes formative mastery	≥80% to complete	Per-item feedback only	Full item analysis + internal key
2	Scenario-based assignments (per module)	Applied case-management judgment	Submission + rubric/faculty where graded	Rubric feedback	Submissions + scores

Layer	Instrument	Purpose	Threshold	Learner sees	Admin/faculty sees
3	Capstone documentation package (Section I)	Integrated competency	Meets rubric minimum	Rubric feedback	Full rubric scoring
4	Final cumulative assessment (Section H)	Summative competency	≥80% to pass	Pass/fail + remediation routing	Item analysis + internal key

Remediation guidance (after a failed attempt):

- 1 Show targeted feedback identifying weak objective areas (no answer key).
- 2 Auto-route to the relevant lesson(s) for review.
- 3 Require a short reflection or re-do of the related exercise before retry (final exam).
- 4 After maximum attempts, lock and require faculty/SME review and a documented remediation plan.

Answer-key handling: No final answer key is ever shown to learners. A separate **internal answer key** (in the workbook Assessment_Blueprint sheet, admin-only) holds correct answers + rationales for faculty/administrator use.

H. Final Assessment Blueprint

- **Format:** Cumulative final exam drawn from a question bank spanning all 10 modules.
- **Length (recommended):** 40-50 questions (≈4-5 per module), mixed MCQ, multi-select, true/false, and 2-3 scenario items.
- **Bank strategy:** Random draw from a categorized question bank (one category per module) so each attempt differs.
- **Pass mark:** ≥80%.
- **Attempts:** Up to 3; remediation between attempts; lock + faculty review after 3.
- **Security:** Shuffle questions/answers; time limit; **no full-answer review**; only pass/fail + objective-area feedback released.
- **Blueprint coverage map:**

Module	Objectives sampled	Approx. items	Item types
CM-M01	Scope, escalation	4	MCQ, T/F
CM-M02	Assessment, goals, SDOH/bias	5	MCQ, multi-select
CM-M03	Coordination, SBAR	4	MCQ, scenario
CM-M04	Documentation, corrections	5	MCQ, "spot the error"
CM-M05	Rights, consent, ethics	4	MCQ, scenario
CM-M06	Transitions, teach-back	5	MCQ, multi-select
CM-M07	Medical necessity, payer-safe	4	MCQ
CM-M08	QA, incident, escalation	4	MCQ, scenario
CM-M09	Risk stratification, geriatric	5	MCQ, multi-select
CM-M10	Integration	2	scenario

- **Internal key:** Maintained admin-only in the workbook. [[SME REVIEW REQUIRED for final item validity]]

I. Capstone Activity Blueprint

- **Capstone task ID:** CM-CAP-01

- **Scenario (fictional):** An end-to-end home-health case for a fictional, de-identified high-risk client from intake through safe transition.
- **Learner deliverable - integrated documentation package:**
 - 1 Intake + nursing assessment summary (CM-M02)
 - 2 Patient-centered care plan with measurable goals (CM-M02)
 - 3 Interdisciplinary coordination map + SBAR (CM-M03)
 - 4 Audit-ready chart note (CM-M04)
 - 5 Consent/advocacy + ethics reflection (CM-M05)
 - 6 Discharge/transition plan with teach-back (CM-M06)
 - 7 Medical-necessity rationale (CM-M07)
 - 8 Incident/escalation note (CM-M08)
 - 9 Risk-stratification + monitoring plan (CM-M09)
- **Rubric criteria (each scored):** clinical accuracy; nursing-scope appropriateness; documentation quality/audit-readiness; coordination/communication; advocacy/ethics & bias-awareness; transition safety; completeness/integration.
- **Passing standard:** Meets minimum on **every** rubric criterion (no zero criteria); overall $\geq 80\%$ rubric score.
- **Documentation expectation:** Professional, objective, complete, de-identified, internally consistent across artifacts.
- **PHI safety note:** Instructions explicitly forbid real cases; submission carries no-PHI attestation; faculty performs manual PHI review before scoring.
- **Evidence artifact:** Capstone submission + rubric scoring record + faculty PHI-review attestation.
- **SME/compliance flags:** Rubric and scenario require SME sign-off. [[SME REVIEW REQUIRED]]

J. Certificate Gate and Completion Logic

A certificate is issued **only** when all conditions are met (no blank certificates):

Gate item	Required condition	LMS evidence source	Manual review?	Risk if omitted
All modules complete	CM-M01...CM-M10 activity completion = done	Activity completion report	No	Incomplete instruction; contact-hour claim invalid
Knowledge checks passed	Each KC $\geq 80\%$	Gradebook	No	Mastery not demonstrated
Capstone meets rubric	Rubric minimum met on all criteria	Assignment grade	Yes (faculty)	Competency not verified
Final exam passed	$\geq 80\%$	Quiz grade	No	Summative competency not met
Evaluation submitted	Course evaluation complete	Feedback activity	No	Missing required evaluation evidence
Time/participation	Completion timestamps + attempt durations present	Logs/completion	Spot-check	Weak time-on-task evidence
No-PHI attestation	Learner attestation + faculty PHI review (capstone)	Assignment attestation	Yes	Privacy exposure
Approval placeholder	CEP number field shows placeholder until issued	Certificate template	Yes	Misrepresentation of approval

Completion logic: Course completion (all activity criteria + final pass + evaluation) → unlocks the **gated Moodle Certificate** activity → certificate generates with the placeholder CEP line until a real number is issued.

K. Draft Course Information Form Support

Field values below are **prepared from the master packet** for the *new 30-hour course*; items not in the packet are marked **Needs confirmation**.

BRN Course-Information field	Draft value	Source / status
Provider/Business Name	CI Institute of Nursing	Master packet (BRN_TRANSFER_WORKSHEET)
Course Title	RN Case Management in Home Health: Care Coordination, Documentation, Patient Advocacy, and Safe Transitions of Care	This package (new 30-hr design) [[SME REVIEW]]
Date(s) to be Offered	Needs confirmation (schedule item)	Not source-locked
Objectives (behavioral)	See Section E (measurable objectives per module)	This package
Overview/Description	See Section M / Section A	This package
Type of Offering	Online self-paced via Moodle LMS (asynchronous, time-tracked)	This package - differs from packet's "live webinar" representative course [[SME/COMPLIANCE REVIEW]]
Teaching Methods	Reading, recorded/branching lessons, scenarios, knowledge checks, documentation exercises, capstone, final exam	This package
Number of Contact Hours	30 contact hours (10 modules x 3; 50 min = 1 contact hour; 1,500-min minimum, 1,680 design minutes)	This package; official map + transfer worksheet aligned to 30 contact hours
Independent-study methodology	Applies: self-paced online; time-on-task via LMS logs, gated sequencing, ≥80% assessments	This package
Content (outline form)	See Section D + Section E	This package
Implicit-bias incorporation	Bias reflection embedded in Modules 2, 5, 9 and capstone	This package (mirrors packet field)
Method of Evaluation	KCs + assignments + capstone rubric + final exam ≥80% + end-of-course evaluation	This package

Offering-type reconciliation flag: Contact hours are reconciled at **30 contact hours** in both the official BRN contact-hour map / transfer worksheet and this package. The official transfer worksheet currently lists a **live webinar / live-stream** delivery mode, while this package is designed for **online self-paced** delivery. The delivery mode (not the contact-hour count) must be confirmed by SME/compliance before any BRN submission. [[SME REVIEW REQUIRED BEFORE SUBMISSION]]

L. Draft Instructor Information Checklist

Instructor of record from packet: **Vanessa Valerio, RN License 788389 (License Type: RN)**.

Item	Status	Source / action
Instructor name	Provided	Vanessa Valerio (packet)
License number	Provided	788389 (packet, CDPH E279)
License type	Provided	RN (packet)
License expiration date	Needs confirmation	Vanessa evidence packet - attach
Education	Needs confirmation	Vanessa evidence packet - attach
Experience (clinical)	Needs confirmation	Vanessa evidence packet - attach
Teaching experience	Needs confirmation	Vanessa evidence packet - attach
Adult-education course info	Needs confirmation	Vanessa evidence packet - attach
SME sign-off on 30-hr content	Pending	This package adds 26 hrs beyond the representative course

Item	Status	Source / action
Subject-matter relevance to RN practice	Draftable	Section C rationale

Confirm whether Vanessa Valerio is also the SME for the expanded 30-hour scope or whether an additional SME is needed for high-scrutiny modules (CM-M07, CM-M08). **Needs confirmation.**

M. Draft Flyer / Brochure Copy

Draft - Pending BRN CEP Approval Pending California Board of Registered Nursing Continuing Education Provider approval. Do not advertise, issue certificates, or represent this course as approved until a BRN CEP number has been issued.

CI Institute of Nursing 419 E. Hamilton Ave., Campbell, CA 95008 · 650-449-6706 · humanresources@ci-ion.com · <https://www.ci-ion.com>

Course title: RN Case Management in Home Health: Care Coordination, Documentation, Patient Advocacy, and Safe Transitions of Care

Who it's for: California Registered Nurses working or preparing to work in care management, case management, home-health coordination, intake coordination, discharge planning, quality review, or patient advocacy.

Course description: A practical, fully online continuing education course for RNs in care management. Across ten modules you will strengthen nurse-led care coordination, patient-centered assessment and care planning, audit-ready documentation, patient advocacy and ethics, safe transitions and discharge planning, utilization review, and quality/risk management - applied through fictional, de-identified case scenarios and documentation practice.

Learning objectives (summary): Coordinate care across disciplines and settings; document to a defensible, audit-ready standard; protect patient rights and advocate ethically; plan safe transitions; support medical necessity in nursing terms; and manage high-risk, chronic, and geriatric patients.

Draft contact hours: 30 contact hours (*draft course-design math; not "approved for" any hours until a BRN CEP number is issued*).

Delivery method: Online, self-paced via Moodle LMS, with time-tracked activities and an 80% pass requirement.

Completion requirements: Complete all 10 modules, pass all knowledge checks and the final exam at ≥80%, submit the capstone, and complete the course evaluation.

Refund / cancellation policy (placeholder - see Section P): Written cancellation honored through the first session or the 7th calendar day after enrollment, whichever is later; pro-rata refund when ≤60% of the course is completed; refunds within 45 days. (*Final wording per provider policy.*)

Important notices:

- This course does **not** grant a case management certification, CCM certification, social-work license credit, or any scope expansion. It is RN continuing education only.
- No PHI:** All examples are fictional and de-identified. Do not submit real patient information.

N. Draft Certificate Text

Draft certificate - Pending BRN CEP Approval. Not for issuance until a CEP number is issued. No blank certificates.

CI INSTITUTE OF NURSING
Certificate of Completion - Continuing Education for Registered Nurses

This certifies that

[Learner Full Name]

has successfully completed

RN Case Management in Home Health: Care Coordination,

Documentation, Patient Advocacy, and Safe Transitions of Care

Completion date: _____

Contact hours: [____ contact hours - placeholder until BRN CEP approval]

Provider: CI Institute of Nursing

BRN CEP Provider Number: [CEP # PENDING - placeholder until issued by the California Board of Registered Nursing. Do not represent as approved until issued.]

Authorized signature: _____ Title: _____

RETENTION NOTICE: Please retain this certificate of completion for FOUR (4) YEARS.

This course is continuing education only. It does not grant a case management certification, CCM certification, social work license credit, or any scope expansion. All instructional cases are fictional and de-identified.

Required elements present: learner name · course title · completion date · provider name placeholder · BRN CEP number placeholder · contact-hours placeholder · signature/title placeholder · 4-year retention statement · approval-placeholder language · no-certification/no-scope statement. No real provider number is used (none exists in the master packet).

O. Course Evaluation Form

Required for completion (Section J). Delivered as a Moodle Feedback activity. *Pending BRN CEP Approval - for course-quality evidence only.*

Part 1 - Objective achievement (1=strongly disagree ... 5=strongly agree):

- 1 The stated learning objectives were met.
- 2 Content was relevant to my registered-nursing practice in care/case management.
- 3 The contact-hour/time commitment matched the workload.
- 4 Scenarios and exercises improved my applied judgment.
- 5 Documentation practice was useful and realistic.
- 6 The course was free of commercial bias.
- 7 Materials were accessible and easy to navigate (mobile included).

Part 2 - Instruction & design:

- 1 The instructor/SME content was clear and credible.
- 2 Assessments fairly measured the objectives.
- 3 Feedback/remediation was helpful.

Part 3 - Open response:

- 1 What was most valuable?
- 2 What would you improve?
- 3 Any concerns about accuracy, accessibility, or content?

Part 4 - Attestation: "I confirm I completed the activities myself and did not submit any real patient information (no PHI)." (required checkbox)

P. Recordkeeping and Audit Evidence Plan

Retention: Provider/CE records retained **≥4 years** (institutional posture: 5 years; transcripts indefinitely) at 419 E. Hamilton Ave., Campbell, CA 95008. Records custodian: **Yadvir Saandal** (per master packet). Source: POLICIES_PACKET .md.

Certificate policy: No blank certificates; issued only after completion criteria met; learners instructed to retain for 4 years.

Audit evidence captured per learner:

Evidence item	Generated by	Stored where	Retention	Review owner	Export method
Activity completion report	Moodle completion	LMS + secured backup	≥4 yr	Records custodian	CSV/PDF export
Knowledge-check scores	Moodle gradebook	LMS	≥4 yr	Faculty/admin	Gradebook export
Assignment/exercise submissions	Moodle Assignment	LMS	≥4 yr	Faculty	Bulk download
Capstone + rubric scoring	Moodle Assignment	LMS	≥4 yr	Faculty/SME	PDF export
Final-exam attempt records	Moodle Quiz	LMS	≥4 yr	Faculty/admin	Quiz report export
Time-on-task logs	Moodle logs	LMS	≥4 yr	Admin	Logs export
Evaluation responses	Moodle Feedback	LMS	≥4 yr	Admin	Feedback export
Issued certificate record	Certificate activity	LMS + records file	≥4 yr	Records custodian	PDF + register
No-PHI attestations	Assignment attestation	LMS	≥4 yr	Faculty	Export

Export cadence: Per-learner completion package exported on certificate issuance; full-course audit export quarterly. **No PHI** stored in any evidence artifact.

Q. Supporting XLSX Workbook Plan

File: BRN_CM_CE_LMS_BUILD_WORKBOOK .xlsx - a build-support and defensibility artifact only; the markdown package is the primary deliverable. Sheets:

- 1 Course_Map - Module ID, title, contact hours, est. minutes, BRN content rationale, direct/indirect linkage, completion requirement, evidence artifact.
- 2 Module_Blueprints - Module ID, lesson ID, lesson title, learning objective, activity type, est. minutes, scenario/exercise summary, Moodle activity recommendation, completion condition.
- 3 LMS_Activity_Map - Activity ID, Module ID, activity name, Moodle activity type, learner action, completion-tracking setting, restrict-access rule, gradebook category, evidence generated.
- 4 Assessment_Blueprint - Assessment ID, Module ID, question type, question stem, answer choices, **correct answer**, rationale, remediation guidance, learner visibility, admin/faculty visibility (internal key - admin only).
- 5 Capstone_Map - Capstone task ID, scenario summary, learner deliverable, rubric criterion, passing standard, documentation expectation, PHI safety note, evidence artifact.
- 6 Application_Packet_Checklist - Packet item, draft status, owner, source reference, missing information, approval status, notes.
- 7 Certificate_Gate - Gate item, required condition, LMS evidence source, manual review needed, approval placeholder, risk if omitted.
- 8 SME_Compliance_Flags - Item ID, Module ID, flag type, issue, risk level, reviewer needed, required action, status.
- 9 Audit_Evidence_Log - Evidence item, generated by, stored where, retention period, review responsibility, export method, notes.

The workbook is generated by `build_workbook.py` (same folder) and mirrors this markdown; the markdown governs if they ever differ.

R. Compliance Review Checklist

#	Compliance item	Status	Reviewer
R1	Placeholder approval language on every learner-facing artifact	Drafted	Compliance
R2	"Contact hours," not "CEUs"; no "approved for" claims	Drafted	Compliance
R3	Each module ≥ 150 min; none < 60 min; 30 hr total math correct	Verified in design	Compliance
R4	Certificate carries 4-yr retention; no blank certificates	Drafted	Compliance/Legal
R5	Records retained ≥ 4 yr; custodian named	Drafted (packet)	Records custodian
R6	No PHI anywhere; all cases fictional/de-identified	Designed-in	Compliance/Privacy
R7	No certification/scope-expansion claims	Drafted	Compliance/Legal
R8	80% threshold; no learner answer key; internal key separated	Designed-in	Faculty/Admin
R9	Offering-type & contact-hour discrepancy vs. packet reconciled	Open	SME/Compliance
R10	Implicit-bias content present and BRN-aligned	Drafted	SME
R11	Accessibility (WCAG 2.1 AA) requirements specified	Drafted	Accessibility
R12	Refund/cancellation/recordkeeping match packet policy	Drafted (packet)	Legal

S. SME Review Checklist

#	SME item	Module(s)	Status
S1	RN scope-of-practice statements accurate and current	M01	Pending SME
S2	Assessment frameworks & care-plan template current	M02	Pending SME
S3	Handoff/SBAR examples reflect practice	M03	Pending SME
S4	Documentation/legal-charting standards correct	M04	Pending SME + Legal
S5	Consent/advocacy/ethics content correct	M05	Pending SME + Legal
S6	Discharge/teach-back content correct	M06	Pending SME
S7	Medical-necessity content stays in nursing lane	M07	Pending SME + Compliance
S8	QA/risk/incident/escalation patient-safety framed	M08	Pending SME + Compliance
S9	Chronic/geriatric clinical content current & education-only	M09	Pending SME
S10	Capstone rubric & final-exam items valid and fair	M10	Pending SME

#	SME item	Module(s)	Status
S11	Confirm SME-of-record for full 30-hr scope (Vanessa Valerio or +1)	All	Pending
S12	All scenarios verified fictional/de-identified	All	Pending SME

T. Open Questions / Missing Packet Items

Items not found in the master packet (do not invent - confirm before submission):

- Offering-type reconciliation:** Contact hours are reconciled at **30 contact hours** across 02_BRN_CEP_COURSE_AND_CONTACT_HOUR_MAP .md, BRN_TRANSFER_WORKSHEET .md, and this package. Delivery mode still differs (worksheet: live webinar / live-stream; this design: online self-paced). Confirm the intended delivery mode before submission. [[SME REVIEW REQUIRED]]
- Offering type/independent-study:** If online self-paced, confirm BRN independent-study methodology requirements are satisfied.
- Instructor evidence (Vanessa Valerio):** license expiration, education, experience, teaching experience, adult-education info - all **Needs confirmation** (per 06_MISSING_ITEMS .md).
- SME of record for expanded scope** (esp. M07/M08) - **Needs confirmation**.
- Prior California nursing CE provider history** answer - **Final verification** (per packet).
- CE coordinator designation** - **Final verification** (per packet).
- Course offer dates / schedule** - **Needs confirmation**.
- Legal/privacy sign-off** on policy + certificate wording - **Pending** (per packet).
- \$750 payment/check attachment** confirmation - **Pending** (per packet).
- BRN CEP number** - **Not issued**. All approval language stays placeholder until issued.
- Final exam & KC item bank** - items are blueprinted here; full item text + validated internal key require SME authoring/sign-off.

Items confirmed from packet (source of truth): provider identity, address/phone/email/website, FEIN, corporation status, records custodian (Yadvir Saandal), instructor of record (Vanessa Valerio, RN 788389), refund/cancellation/recordkeeping policy, certificate required elements + 4-yr retention, \$750 fee.

U. Final Build Handoff Checklist

#	Handoff step	Owner	Gate
U1	Approve 30-hr scope & reconcile vs. 4-hr packet course	Exec + SME	Blocks BRN submission
U2	SME authors/validates KC + final-exam items + internal key	SME	Blocks go-live
U3	Build Moodle shell (10 sections + Start Here + Certificate) per Section F	LMS admin	-
U4	Configure completion, restrict-access, quiz, gradebook, certificate gate	LMS admin	-
U5	Load fictional scenarios/templates; run no-PHI audit	LMS admin + Compliance	Blocks go-live

#	Handoff step	Owner	Gate
U6	Accessibility QA (WCAG 2.1 AA) + mobile QA	Accessibility	Blocks go-live
U7	Legal/privacy sign-off on certificate, flyer, policies	Legal	Blocks advertising
U8	Attach instructor evidence (Vanessa Valerio)	Records custodian	Blocks BRN submission
U9	Confirm \$750 payment artifact	Finance	Blocks BRN submission
U10	Submit BRN CEP application; await CEP number	Authorized signer	-
U11	After CEP number issued: replace all placeholders, enable certificate, enable advertising	Compliance + LMS admin	Final
U12	Pilot cohort + evaluation review before full launch	Faculty	Recommended

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